APPENDIX A · PILOT EVIDENCE

October Cancer Screening Initiative

OCSI 2025

Pilot Summary · Key Findings · Lessons for EVAH

TAHA · Nazareth Hospital · Limuru Sub-County

759

women sensitized & prescreened

119 screened at Nazareth

45–50 abnormal findings

34.7% of screened were age 45–54

Direct precursor to the EVAH Pathway A project →

What OCSI Was & Who It Involved

BACKGROUND + PARTNERS

OCSI was a community-to-hospital early detection initiative testing whether structured community mobilization could be reliably linked to hospital-based cancer screening and follow-up — creating a traceable, low-cost rural pathway.

1 Church-Based Prescreening

CHPs mobilized & prescreened women in Ngecha and Riara Ridge via paper or mobile forms

2 SMS + Navigation Support

Selected women received SMS comms, booking support & navigation. All prescr
women could access subsidized screenin

Implementation Partners

Tujali Afya
HEALTH ALLIANCE

Community
Health Services
"Afya Yetu, Jukumu Letu"

Community Health Services
Limuru Sub-County Public Health Office

Objectives

Goal: Improve early detection of breast and cervical cancer among rural women in Kiambu County

500–1,000

Women to reach

Pre-screening and health education through Community Health Promoters and organized mobilization events in rural villages

100

Screening vouchers

Need-based vouchers for breast and cervical cancer screening at Nazareth Hospital, targeting the most underserved women

1

Scalable model

Build an equitable, low-cost, replicable model linking community prescreening to hospital-based screening and follow-up

Key Results

OCSI 2025 achieved above-target reach — and exposed the continuity gap the EVAH project will address

759
Women Sensitized & Prescreened

119
Completed Screening at Nazareth Hospital

45–50
Abnormal Findings Identified

KES 600
Subsidized Screening Fee Per Patient

Target: 50 screened **Achieved: 119 (+138%)**

Target: 500–1,000 sensitized **Achieved: 759 ✓**

PATHWAY FUNNEL

759
Sensitized & Prescreened

→

119
Screened at Nazareth

→

The Continuity Gap

Only 15.7% of prescreened women completed screening at Nazareth. The 84% pathway loss is the central problem the EVAH project addresses.

Reach & Age Distribution

OPERATIONAL DATA

Top 10 Regions: Prescreened vs Screened

Summary of Prescreened and Screened Clients for the Top 10 Regions

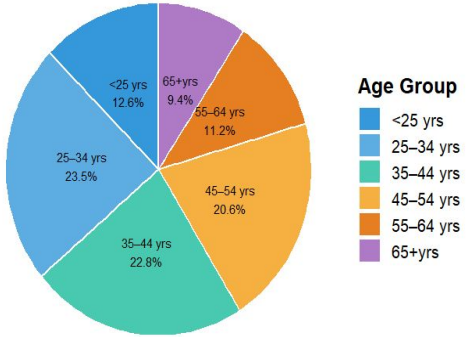
Region	Prescreened Clients	% of Prescreened	Screened Clients	% of Screened
Ngecha	274	36.1%	18	15.0%
Karanjee	89	11.7%	28	23.3%
Kabuku	84	11.1%	1	0.8%
Kawaida	56	7.4%	25	20.8%
Riara	47	6.2%	17	14.2%
Itungi	55	7.2%	1	0.8%
Gatono	17	2.2%	7	5.8%
Ruthui	22	2.9%	0	0.0%
Nyathuna	12	1.6%	1	0.8%
Gitangu	8	1.1%	0	0.0%

Percentages represent each region's contribution to the total number of prescreened and screened clients.

Age Profile of Screened Women

Age Distribution of Prescreened Clients

Percentage of clients prescreened by age group



34.7% of screened women were aged **45–54 years**

Middle-aged women showed highest uptake — younger age groups remain under-reached

Voucher Redemption

VOUCHER SYSTEM DATA

The voucher system provided targeted access for underserved women. Redemption data reveals both the reach of the voucher model and the persistence of transport and access barriers that prevented higher uptake.

108

Total Vouchers Issued

54

Total Vouchers Redeemed

50%

Redemption Rate

Total Vouchers: Issued vs Redeemed

Summary of Total Processed Vouchers vs Redeemed

Total Vouchers	Total Redeemed
108	54

Top 5 Villages: Issued, Redeemed & Unredeemed

Top 5 Villages: Issued, Redeemed, and Unredeemed Vouchers

Village	Issued	Redeemed	Issued but NOT Redeemed
Ngecha	19	6	13
Karanjee	11	20	-9
Itungi	7	1	6
Riara	5	7	-2
Kawaida	4	11	-7

Barriers to Screening Uptake

IMPLEMENTATION LESSONS

OCSI revealed five persistent barriers that explain the gap between prescreening and completed care — each directly motivating components of the proposed EVAH intervention.

Transport Cost

Even with screening vouchers, low redemption persisted due to transport costs to Nazareth. Financial vulnerability and cancer risk were closely linked.

Layered Vulnerability

Those with vouchers (higher risk) were less likely to complete screening than those paying out-of-pocket — highlighting equity concerns.

Fear & Stigma

Fear of screening procedures and uncertainty about results led some women to prefer not knowing their status. Some refused VIA/VILI.

Low Awareness

Limited awareness of the importance of screening and early detection of cancer among target populations reduced proactive health-seeking.

Navigation Barriers

Some women were unaware of screening services at Nazareth or found the referral pathway confusing. Access required reducing bureaucracy.

What OCSI Showed — and What It Exposed

✓ WHAT WORKED

- Community-based mobilization for cancer screening is feasible
- Digital and paper workflows can function in frontline settings
- Hospital-linked screening can be organized through a structured rural pathway
- Real-world implementation data can be generated at community and facility levels
- Subsidized screening (KES 600) creates meaningful affordability for rural women

⚠ WHAT IT EXPOSED

Major losses between identification and completed care

- **Transport cost:** Even voucher holders couldn't always get to Nazareth
- **Fragmented workflow:** No structured booking, reminders, or follow-up system
- **Weak follow-up:** Abnormal findings not systematically tracked post-screening
- **Documentation gaps:** Paper-heavy process made continuity hard to maintain
- **Stigma & fear:** Prevented women from returning even after initial sensitization

Why This Matters for EVAH

OCSI is the direct evidence base and operational precursor for the proposed Pathway A evaluation

OCSI 2025
What we proved

- Community mobilization is feasible
- Prescreening at scale (759 women)
- Hospital linkage works
- Digital workflows are viable
- Continuity gap is real & measurable

EVAH builds on this

The EVAH Pathway A Question

Can a community-proximate, fixed-cadence HealthPod model — supported by a bounded AI-enabled workflow layer — reduce the leakage observed in OCSI and create a more reliable pathway to completed, traceable care?

- 5 CHUs · stepped-wedge rollout
- Monthly HealthPod 'Cancer Week' sessions
- Bounded AI workflow: booking, reminders, escalation
- Nazareth Hospital as clinical anchor
- 12-month Pathway A implementation study

Field Experience

IMPLEMENTATION IN ACTION

OCSI demonstrated strong teamwork and efficient partner coordination across community and facility levels. Digital data collection was adopted successfully — validating the readiness of frontline actors to use mobile-accessible tools.

Community prescreening — Ngecha & Riara Ridge

Partner field team coordination — TAHA / CHPs

Tujali Afya Health Alliance

Leading community-proximate cancer early detection in Kenya

759	119	45–50	5	1
Prescreened	Screened	Abnormal	Barriers ID'd	Pathway gap ⟶ EVAH

OCSI proved that community mobilization and hospital linkage are feasible in rural Kiambu County. It also exposed the continuity gap — the 84% of prescreened women who did not complete care — that the EVAH Pathway A project is designed to address through a structured, community-proximate, AI-enabled continuity model.

Supporting documentation: OCSI presentation deck, workflow materials, and implementation data available on request.